DISCHARGE SUMMARY

FICTIONAL DOCUMENT - SYNTHETIC DATA ONLY - NOT FOR CLINICAL USE

Patient details

Full name:	Robert Ellis (fictional)
Date of birth:	14 March 1957 (age 69)
MRN:	FICTIONAL-001
Ward:	Orthopaedic Ward 7B, Riverside Demo Hospital (fictional)
Admitted:	21 October 2026
Discharged:	24 October 2026
Primary contact:	Margaret Ellis (wife) - caregiver

Procedure and diagnosis

Procedure:	Left total hip arthroplasty (cemented)
Surgeon:	Dr. A. Nkosi (fictional), Orthopaedic Surgery
Anaesthetic:	Spinal anaesthesia with sedation
Diagnosis:	Primary osteoarthritis of the left hip, end-stage
Implant:	Details on file with the surgical team (not reproduced here)

Discharge status

Mr Ellis was discharged in a clinically stable condition following an uncomplicated left total hip arthroplasty. He is ambulatory with a walking frame. Oral analgesia has been commenced. All post-operative observations were within normal limits at the time of discharge. The patient was accompanied home by his wife.

IMPORTANT: This discharge summary was prepared under time pressure at the end of a busy shift. Some sections contain provisional or incomplete information. Caregivers with questions should contact the surgical team or GP rather than acting on ambiguous instructions without clarification.

Section 2 - Medications

2.1 Prescribed anti-inflammatory (pain management)

Mr Ellis has been prescribed an oral non-steroidal anti-inflammatory drug (NSAID) for post-operative pain management. The specific product and dose are documented on the pharmacy dispensing label attached to the medication packaging.

Dosing schedule: take as required when pain is present, OR on a regular scheduled basis as directed by the dispensing pharmacist.

Starting dose: adjust per individual tolerance and response. Do not exceed the maximum daily dose stated on the product label.

AMB-1: The name of the prescribed NSAID is NOT recorded in this document. Refer to the pharmacy label on the medication box for the drug name and dose.

AMB-2: Dosing schedule is contradictory - 'as required' and 'regular scheduled basis' are both stated without priority guidance. No specific frequency is defined.

2.2 Paracetamol (adjunct analgesia)

Paracetamol 500 mg to 1 g may be taken as needed for additional pain relief. Frequency: every 4 to 6 hours as required.

Advisory: The combined daily maximum dose when paracetamol is taken alongside the prescribed NSAID is not specified in this document. Consult the pharmacist.

2.3 Existing home medications

Continue ALL existing home medications unless otherwise instructed by the surgical team or GP.

AMB-5: No list of existing home medications is included in this discharge summary. 'Continue all existing medications' cannot be verified without a medication reconciliation record. The patient reports taking daily low-dose aspirin for a cardiac condition.

AMB-6: Potential interaction between daily low-dose aspirin and the prescribed NSAID was raised during the ward round but was NOT reviewed or resolved at the time of discharge. This interaction was acknowledged by the clinical team with the note 'we should check that'. No resolution or interim guidance is documented.

2.4 Weaning off anti-inflammatories

The prescribed NSAID should be gradually reduced and discontinued as recovery progresses and pain subsides.

Advisory: No weaning timeline or schedule has been provided. Duration of NSAID therapy is not defined in this document.

Section 3 - Wound Care

3.1 Initial dry period

The surgical wound must be kept completely dry for the first 48 hours following discharge.

3.2 Showering and bathing

The patient may take a light shower after day 3 post-discharge provided the wound area is adequately covered with a waterproof dressing.

Do not submerge the wound site in water (baths, swimming pools) until the wound is fully healed and the skin is intact.

AMB-8: Conflicting dry-period instructions. Section 3.1 states 48 hours; section 3.2 states 'after day 3' (72 hours). The correct window is not clarified.

Advisory: The type of waterproof dressing required for showering is not specified. The discharge pack contents are listed on a separate sheet not included here.

3.3 Dressing changes

Perform daily dressing changes until the wound is fully healed. Dressing materials have been included in the discharge pack provided at the ward.

Do not disturb the primary wound dressing unless it becomes soiled or saturated. If in doubt, contact the district nursing team.

AMB-10: Contradictory dressing guidance. 'Daily dressing changes' (paragraph 1) conflicts directly with 'do not disturb unless soiled' (paragraph 2). No priority or clarification is given.

3.4 Wound monitoring

Inspect the wound at each dressing change. Contact the surgical team or attend the emergency department if any of the following are observed:

- Redness, increasing warmth or swelling around the wound
- Any discharge (clear, yellow or blood-stained)
- Wound edges separating or sutures/clips appearing loose
- Fever above 38.0 degrees C
- Increasing rather than decreasing pain after the first 48 hours

Advisory: No district nursing referral is confirmed in this document. It is unclear whether a community nurse visit has been arranged for the first dressing change.

Section 4 - Mobility and Activity

4.1 Immediate post-discharge rest

Strict bed rest is recommended for the first 24 to 48 hours following discharge to allow the surgical site to stabilise.

4.2 Early mobilisation

Early mobilisation is strongly encouraged to reduce the risk of complications including deep vein thrombosis (DVT), chest infection and pressure injury. Short, gentle walks with the walking frame should begin as soon as the patient feels able to do so.

AMB-12: Direct contradiction. Section 4.1 recommends 'strict bed rest for 24-48 hours'. Section 4.2 recommends 'early mobilisation as soon as able'. No guidance is given on how to reconcile these instructions or which takes priority.

4.3 Hip precautions

Standard hip precautions must be maintained throughout the recovery period. Refer to the physiotherapy instruction sheet provided on the ward.

AMB-13: Hip precautions are referenced but not explained in this document. The physiotherapy instruction sheet is not included in this discharge summary. It is unknown whether the patient received or retained this sheet.

4.4 Weight bearing

Weight bearing: as tolerated, using the walking frame provided.

Note from physiotherapy assessment: protected weight bearing recommended until reviewed at follow-up. Do not progress beyond partial weight bearing without physiotherapist review.

AMB-14: Weight bearing instructions conflict. Main text states 'as tolerated'; physiotherapy note states 'protected/partial weight bearing only until review'. No reconciliation or clinical decision is documented.

4.5 Stairs

Stair use is permitted with caution once the patient has adequate strength and confidence. Always use the handrail and take one step at a time.

Avoid stairs where possible during the initial recovery phase.

AMB-15: Conflicting stair guidance. 'Permitted with caution' contradicts 'avoid where possible during initial recovery'. No definition of when the initial recovery phase ends.

4.6 Physical therapy

A physiotherapy referral has been submitted. Community physiotherapy will make contact to arrange an initial assessment. Begin gentle range-of-motion exercises when the patient feels comfortable and ready.

Advisory: No start date or minimum frequency for physiotherapy has been defined. The timeframe for community physiotherapy to make contact is not stated. It is not confirmed whether the referral has been accepted.

4.7 Driving

Mr Ellis must not drive until he is able to perform an emergency stop safely and has received medical

clearance from his GP or the surgical team.

Advisory: No minimum recovery period before driving assessment is stated. The criteria for 'medical clearance' are not defined in this document.

4.8 Return to work and domestic activities

Light domestic activities are permitted when the patient feels comfortable. Return to work should be discussed with the surgeon at the follow-up appointment. Heavy lifting (over approximately 5 kg) should be avoided until cleared by the surgical team - no specific timeframe is given.

Advisory: Alcohol is not addressed in this discharge document. The patient or caregiver should seek pharmacist or GP advice regarding alcohol use while taking the prescribed NSAID and any other medications.

Section 5 - Follow-up Appointments

5.1 Surgical follow-up

Attend a follow-up appointment at the Orthopaedic Outpatient Clinic in approximately 4 to 6 weeks post-discharge.

A written appointment confirmation will be sent by post to the patient's registered home address. No specific date or time was confirmed at the point of discharge. If the letter has not arrived within 10 days, contact the outpatient booking team at the number on your clinic letter.

AMB-20: Conflicting follow-up window. This section states '4 to 6 weeks'; the document footer states 'routine follow-up: 6 weeks minimum'. No clarification is given on which applies.

AMB-21: Contact instructions are circular. The instruction to call 'the number on your clinic letter' assumes the letter has been received - but this instruction is only needed when the letter has NOT arrived.

5.2 GP review

Mr Ellis should see his GP within 2 weeks of discharge if he has any concerns or if recovery is not progressing as expected.

Advisory: No criteria are defined for what constitutes 'concerns' or expected recovery progress. The GP has not been formally notified of this discharge in this document - it is unclear whether a GP discharge letter has been sent.

5.3 Blood tests

Routine blood tests are required at the follow-up appointment. Fasting may be required - await further instruction from the clinic.

AMB-23: Blood test fasting requirements are unconfirmed. 'Await further instruction from the clinic' provides no actionable guidance before the appointment day.

Section 6 - Emergency Guidance

6.1 When to seek urgent help

Return IMMEDIATELY to the emergency department or call emergency services if experiencing any of the following:

- High fever (above 38.5 degrees C)
- Severe or rapidly worsening pain not responding to prescribed medication
- Sudden significant swelling of the operated limb
- Signs of wound infection (redness, heat, discharge, unpleasant odour)
- Calf pain and/or swelling in either leg (possible DVT)
- Shortness of breath or chest pain (possible pulmonary embolism)
- Dislocation or suspected dislocation of the hip implant

6.2 Non-urgent queries

For non-urgent post-discharge questions, contact the surgical team during working hours using the contact details provided on your discharge letter.

AMB-24: No contact telephone number, email address or out-of-hours pathway is provided in this discharge summary. All contact references point to a 'discharge letter' or 'clinic letter' that is separate from this document. It is not confirmed that these letters were given to the patient at discharge.

AMB-24: No contact telephone number, email address or out-of-hours pathway is provided in this discharge summary. All contact references point to a 'discharge letter' or 'clinic letter' that is separate from this document. It is not confirmed that these letters were given to the patient at discharge.

6.3 General welfare

If Mr Ellis appears generally unwell - such as confusion, unusual drowsiness, or significant deterioration in function - seek medical advice promptly.

Advisory: AMB-25: 'Generally unwell' is not clinically defined. No threshold, scoring system or specific symptom criteria is provided for this instruction.

Appendix A - Flagged Ambiguities (For Review)

The following ambiguities were identified in this discharge document. They are reproduced here for clinical review and caregiver reference. This appendix is part of the fictional test record.

AMB-1	Prescribed NSAID name not stated. Only 'the blue ones' referenced verbally.
AMB-2	Dosing: 'as required' and 'regular schedule' both stated, no priority given.
AMB-3	Starting NSAID dose not specified - 'adjust per tolerance'.
AMB-4	Combined daily maximum for NSAID + paracetamol not calculated or stated.
AMB-5	Home medication list not included. 'Continue all' is unverifiable.
AMB-6	Aspirin/NSAID interaction raised but not resolved. No interim guidance.
AMB-7	NSAID wean: no timeline, no schedule, no stopping criteria.
AMB-8	Wound dry period: 48 h (Sec 3.1) vs day 3 / 72 h (Sec 3.2).
AMB-9	Waterproof dressing type for showering not specified.
AMB-10	Dressing changes: 'daily' conflicts with 'do not disturb unless soiled'.
AMB-11	District nursing referral not confirmed in this document.
AMB-12	Mobility: 'strict bed rest 24-48 h' conflicts with 'early mobilisation'.
AMB-13	Hip precautions mentioned but not defined. PT sheet not included.
AMB-14	Weight bearing: 'as tolerated' vs physio note 'protected only'.
AMB-15	Stairs: 'permitted with caution' vs 'avoid where possible'.
AMB-16	PT referral submitted but start date and contact timeline undefined.
AMB-17	Driving: no minimum recovery period or clearance criteria defined.
AMB-18	Return to work: undefined. 'When comfortable' has no clinical threshold.
AMB-19	Alcohol not addressed anywhere in the discharge documentation.
AMB-20	Follow-up window: '4-6 weeks' body vs '6 weeks minimum' footer.
AMB-21	Contact instructions circular: call the number in the letter not received.
AMB-22	GP review criteria not defined. GP notification status unknown.
AMB-23	Blood test fasting: 'may be required, await instruction' - unresolved.
AMB-24	No telephone number or out-of-hours contact in this document.
AMB-25	'Generally unwell' undefined. No criteria for non-emergency escalation.

This document is entirely fictional. All patient names, clinician names, hospital names, MRN numbers and clinical details have been invented for software testing. This document must not be used to guide any real medical decision or action. Generated by PlainSpeak test fixture scripts.